جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : جيهان عبدالحميد محمد احمد
المحافظة : الجيزه **الحي / المدينة :** اوسيم **العنوان :** بشتيل
رقم البطاقة : 27802062101149 **رقم التليفون :** 01123879871 **تاريخ تسجيل الإستمارة :** 17-10-2026
العمر(تاريخ الميلاد) : 06-02-1978 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام تصوير مقطعي للصدر والبطن والحوض بالصبغة [11129] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: جيهان عبدالحميد محمد احمد
National ID: 27802062101149

GUSTAVE ROUSSY INTERNATIONAL EGYPT

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

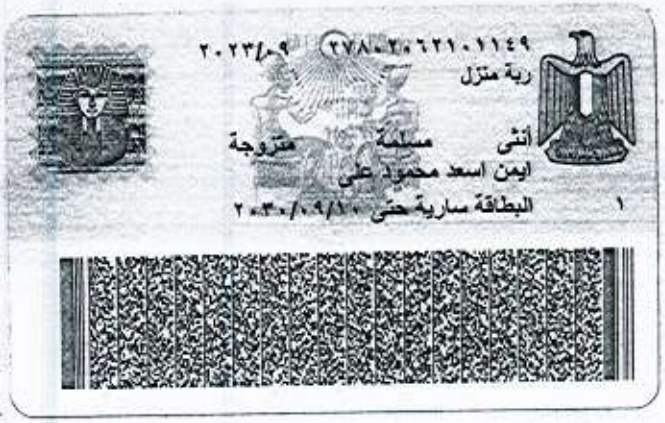
تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced CT of chest, abdomen and pelvis.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

جوستافروسي

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الست المصرية هي صحة مصر

MDT FORM 6/7/2025 G

Name		MRN	3547/24
		ID	
Age	47 yrs.	Phone	01123879871
Menopausal status		Marital status	
NRS SCORE		Family history	
BMI		REASON OF MDT	Post NAC
CO morbidity	m → diab s → left BCS.		

Clinical Data

BX: S/P left BCS, Post NACT

ER:	PR:	HER 2:	Ki67:
+	+	-Ve	

Sonomammography:
→ Residual microcalcific focus of IDC, GII: CTuA CT3a

R L

TNM

Mets Workup:
→ Free margins
→ Free L.N. 0/5
→ Post Pathological mCR

Recommendations:
→ Adjuvant Hormonal therapy + Zoledronic acid
→ Refer to Genetic counseling + Lymphedema

ypTimi; ypN0

MDT OUTCOME & PLAN

Dr: AJ Hulett → AI + Zoledronic acid
+ Refer to Genetic counseling

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Name: جيهان عبدالحميد محمد **MR. :** 2011	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 48-year-old individual with left breast invasive ductal carcinoma, estrogen receptor 8/8, progesterone receptor 8/8, HER2-negative, clinical stage T4aN3M0. Neoadjuvant chemotherapy consisted of 4 cycles of AC followed by 4 cycles of TX, after which the patient underwent left breast conserving surgery and axillary lymph node dissection in May 2025. A port was placed in July 2025 for subsequent therapy. Ongoing management includes sequential hormonal therapy with Femara, Calcium One Alpha, and Zoladex 10.8 mg scheduled for administration on 19 April 2026, with a dental assessment performed for Zometa.

The treatment plan remains pending continuation of the hormonal regimen comprising Zoladex, Femara, Calcium One Alpha, and Zoladex 10.8 mg on 19 April 2026, with a request for a full laboratory work-up prior to the next administration.

The patient is to continue Zoladex and Femara, with the next follow-up appointment scheduled for 10 October, coinciding with the due date of the next Zoladex injection.

تحريرا في ,

التاريخ :	**19-09-2026**	**الوقت :**	

الطبيب المعالج : **ايه مدحت بدوي محمد**

يعتمد ,

Patient Name	: الأستاذة / جيهان عبد الحميد محمد	Request No.	: 13007632
Patient No.	: 27802062101149	Request Date	: 31/07/2024
Pain No.	: 2684/24	Verification Date	: 07/08/2024
Sex	: Female	Financial Category	: اوراد دار السلام - نفقة الدولة
Age	: 46 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: marwa mohamed youss

Pathology Report

Clinical Data: Left breast mass.

Nature of Specimen: Tru-cut bicopsy, collected at 31/7/2024, 11:30 am.

Gross Examination:
- 3 grayish white tissue fragments up to 0.3 cm, totally submitted.

Microscopic Examination:
- Histological type: Invasive duct carcinoma.
- Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- Ductal carcinoma in situ: Focal
- Perineural invasion: No.
- Angioinvasion: No.

Diagnosis:
- Left breast mass, Tru-cut biopsy:
- Invasive duct carcinoma, G II with focal DCIS
- Immunohistochemistry: ER, PR, Her2, Ki67.

Signature
Prof. Dr. Lubna Omar.
Dr Menna Nabil.

Verified by :

Print Date : 07/08/2024 11:17:04

Printing By : Mrs.Mayada Hegazy

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo. Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

ient Name	: الأستاذة / حنان عبد الحميد محمد	Request No.	: 13006793
ient No.	: 27802062101149	Request Date	: 21/05/2025
: No.	: 1850/25	Verification Date	: 03/06/2025
:	: Female	Financial Category	: اوراد غير المسلحة - تغطية كاملة
:	: 47 Y	Department	: Out Patient
erreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

INICAL DATA: Left breast cancer, post NACT .
\TURE OF SPECIMEM: Left BCS
ROSS EXAMINATION:
Procedure: Left BCS
orofatty tissue piece measured 7x6.5x2 cm with covering skin ellipse measured 4x1.5 cm .
Tumor extent : A greyish white tumour bed site
Size: 4.5x2.5x1.5 cm .
Skin involvement: Unremarkable grossly .
Distance from margins: 0.5 cm away from least margin
Frozen section: Free margins
ICROSCOPIC EXAMINATION:
Histological type: Tiny microscopic focus of invasive duct carcinoma (Cellularity 5%).
Maximal tumor diameter (2mmx2mm)
Grade (differentiation+ nuclear pleomorphism + mitosis): G II(3+2+1)
Ductal carcinoma in situ: No
Lymphovascular emboli: Positive
Skin involvement: Free
Resection margins: Free margins
Histologic response to therapy: Pathologic partial response
IAGNOSIS:
eft BCS &SLNB (post NACT) :
Residual microscopic focus of invasive duct carcinoma grade II
Free margins
Free lymph node (0/5)
Pathologic partial response (PPR)
Stage : ypT1mi, yp N0
RCB : 0.947 , RCB class I

Verified by :
Dr.Heba Hasan

عزة رقم (1) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (1) - First Floor-Building No. 987- Nile Corniche - Misr El Qadeam - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

tient Name	: الأستاذه / جيهان عبد الحميد محمد	Request No.	: 13007632
ıtient No.	: 27802062101149	Request Date	: 31/07/2024
ih No.	: 2684/24	Verification Date	: 17/08/2024
:x	: Fema:e	Financial Category	: أورام دار السلام - نفقة الدولة
ıe	: 46 Y	Department	: Out Patient
ıferreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed you

IMMUNOHISTOCHEMISTRY REPORT

nical Data: Left breast mass.
ture of Specimen: Tru-cut biopsy, collected at 31/7/2024, 11:30 am.

chnique:
iections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana)
d stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and
matoxylin as counter stain. The sections treated against:
:R clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

esults:
PT (Positive control showed appropriate reactivity):
:R: Strong nuclear reaction in 80% of the tumor cells (score 8/8).
PR: Strong nuclear reaction in 85% of tumor cells (score 8/8).
Her2: No membrane reaction in thre tumor cells (score 0).
Ki67: 15% nuclear reaction of tumor cells.

onclusion:
eft breast mass, Tru-cut biopsy:
ıvasive duct carcinoma, Grade II with focal DCIS.
PT:
ER: Positive (score 8/8).
PR: Positive (score 8/8).
Her2: Negative (score 0).
Ki67: Positive in 15% of tumor cells.

Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining: 1,

Verified by :

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

tient Name	الأستاذه / جيهان عبد الحميد محمد	Request No.	13007632
tient No.	27802062101149	Request Date	31/07/2024
:h No.	2684/24	Verification Date	17/08/2024
ıx	Female	Financial Category	اورام دار السلام - تغطية الدولة
ıe	46 Y	Department	Out Patient
:ferreing Doc.		Registered by	.marwa mohamed youss

IMMUNOHISTOCHEMISTRY REPORT

%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between %-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and strong). Total score from 0 to 8.
:coring system: "Her2"
:ference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining served or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of ı invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely rceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane ıining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or mplete and circumferential membrane staining that is intense and within < 10% of the invasive tumor lls. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor lls.

Signature
Dr. Walaa Ghonaim.
Dr. Toka Medhat.

Verified by :

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Ministry Of Health
Specialized Medical Centres
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
امانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Jihan Abd Al Hamid Mohamed Ahmed	Date : 15/10/2024
Exam : C.T ABDOMEN &PELVIS	Age : 46 Year(s)
Medical ID: 3547/2024	Pat. ID: 90756

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

- **Reported case of left CBS.**

1. Chest:

- *Heterogenous parenchymal texture of the left breast.*
- *Enlarged left axillary level I, II & III lymph nodes, the largest at level I measures 1.7 x 2.3 cm.*
- *Enlarged right axillary level I & II lymph nodes, the largest at level I measures 2 x 1.6 cm.*
- *Prominent left internal mammary lymph node with preserved fatty hilum, measures about 9 x 6 mm.*
- *Subcentimetric right internal mammary lymph nodes.*
- Scattered bilateral small pulmonary nodules, largest is seen at middle lung lobe measures about 5 mm.
- No pulmonary areas of ground glass attenuation, consolidation or cavitary lesions.
- No CT evidence of pathological mediastinal lymph node enlargement.
- No pleural or pericardial collection, pleural thickening or masses.
- No cardiac gross enlargement.

2. Abdomen & pelvis:

- The liver is enlarged showing homogenous parenchymal attenuation. No sizable focal lesions on non-contrast basis.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal non contrast CT appearance of the spleen, pancreas, kidneys and suprarenals glands.
- Averagely filled urinary bladder showing no sizable diverticular outpouching.

CT Scan
[[DOCTYPE:CT_SCAN]]

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Radiology Department

وزارة الصحة والسكان
هيئة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
الأشعة

Patient Name: Jihan Abd Al Hamid Mohamed Ahmed	Date : 15/10/2024
Exam : C.T ABDOMEN &PELVIS	Age : 46 Year(s)
Medical ID: 3547/2024	Pat. ID: 90756

- *Enlarged uterus with lobulated outline seen extending cranially to the level of the umbilicus and showing mural lesions/fibroids. For US correlation.*
- *Prominent porta hepatis, porto-caval, aorto-caval, para-aortic, bilateral commoin, internal and external iliac lymph nodes, the largest one seen at right external iliac group measures about 1.4 x 1.8 cm.*
- No ascites.
- No sizable worrisome osseous lesions.

Opinion:

- **Scattered bilateral small pulmonary nodules. For follow-up.**
- **No sizable hepatic or osseous deposits on non-contrast basis.**

Dr. Ali Elsisy, MSc.

Signature Prof. Menna Tallah Hanafy
23/10/2024 09:27 ,

Signature Prof. Menna Tallah Hanafy
23/10/2024 09:28 ,

CT Scan
[[DOCTYPE:CT_SCAN]]

وزارة الصحة
أمانة المراكز الطبية المتخصصة
مستشفى معهد ناصر
Nasser Institute Hospital

Oncology Center
مركز الأورام
خطابات أورام

Nuclear Medicine Unit

Patient Name: Gihan Abdel Hameed Mohamed

Date: 7/12/2024

Referring physician: Prof. Dr

^{99m}Tc-MDP BONE SCAN

Desired information: *skeletal surveying to detect multiple lesions.*

Technique: Anterior and posterior whole-body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP:

Report:

- No scientigraphic evidence of bony metastases .
- Heterogenous abnormal radictracer uptake is seen at the spine , more determined at mid dorsal , upper lumbar vertebrae likely of Spondylodegenerative changes ,
  - Osteoarthritic changes are seen at , both shoulders ,both knees
- Both kidneys are normally seen.

Conclusion:

No definite hot spot that couid account for osseous metastatic lesions

Thanks for the referral

Nuclear Medicine Unit

DR. Shoria Mahdy. MD

DR Mohamed Emam

١٣٥١ كورنيش النيل - أغاخان - شبرا - القاهرة - ت : ٢٢٠٣٩١٦٤ - ٢٢٠٣٩٣٥٦ فاكس : ٢٢٠٣٩٢٣٦
1351Korniesh El Nile - Aghakhan - Shoubra - Cairo Tel.: 22039164 - 22039356 Fax : 22039 236

PATIENT NAME: GEHAN ABD ELHAMID MOHAMMED AHMED
GENDER: Female
DATE : 10-Nov-2024 03:04 PM

PATIENT ID: 2020123341
AGE: 46 y
ORDER: 3115457,3116132

CONTRAST ENHANCED SPECTRAL MAMMOGRAPHY AND US

INDICATION:
-left breast mass pathologically proven malignant

TECHNIQUE:
- Contrast Enhanced Spectral Mammography CC and MLO views
- IV non-ionic contrast medium infusion 100 ml of omnipaque

REPORT:
- Contrast Enhanced Spectral Mammography showed bilateral symmetrical nodular back ground parenchymal enhancement.
- Left 5 to 6 o'clock deeply seated high density lesion seen by US as irreguler spiculated heterogenous hypoechoic solid lesion measuring about 35x14 mm with anterior ductal extension and small irreguler nodule measuring about 7x6.6 mm seen about 14 mm anterior to index lesion , mass showing moderate heterogenous mass enhancement , being 7 cm from the nipple
- No right breast sizable suspicious mass or non mass enhancement.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Pathological left ~~axillary level~~ 1,2 and 3 axillary LNS
- Indeterminate level 1 RT axillary LNS
- Small prominant left supraclavicular LNS

OPINION:
-left breast pathologically proven malignancy with extension as described (BIRADS 6 .
- No gross right breast pathological mass or non mass enhancement , only dense adenosis (BIRADS 2)

Refer to clinic

www.Baheya.org
16602
مركز بهية اليرم
Mammogram
DOCTOR MAMMOGRAM

Women's Health Outreach Program

Name: Mrs . Jihan Abdulhameed Mohammed Ahmed
Date: 30-07-2024

Bilateral Full field Digital Mammography

Clinical summary: 46 years old lady with negative family history, and past history of left benign lumpectomy five years ago, complaining of left breast lump of two months duration.

Technique

Medio-lateral oblique and CC views . High resolution ultrasound

Detailed Findings

Right breast

- Extremely dense breast paranchyma that lower the sensitivity of the mammography (ACR density D).
- No masses, asymmetric densities, distortion or suspicious clusters of microcalcifications.
- **Ultrasound** demonstrated heterogeneous breast parenchyma showing exaggerated fibro-glandular element for age and mottled appearance.
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axiliary LNs.

Left breast

- Extremely dense breast parenchyma that lower the sensitivity of the mammography (ACR density D).
- No definite masses, asymmetric densities, distortion or suspicious clusters of microcalcifications.
- **Ultrasound** demonstrated heterogeneous breast parenchyma showing exaggerated fibro-glandular element for age and mottled appearance.
- LOQ, 5 o'clock, outer third, subcutaneous, irregular anti-parallel hypoechoic suspicious mass lesion with moderate vascularity on colour Doppler study measuring 24 x 17 mm. (corresponding to the felt lump)
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Multiple pathological axillary lymph nodes are noted, (more than 7 at level 1), largest measured 28 x 11 mm with 6 mm cortical thickness, largest n level 2 measured 26 x 11 mm with 6.5 mm cortical thickness, and the largest on supraclav. measured 11 x 5 mm with 4 mm cortical thickness.

Impression:

Right breast **(BIRADS 2): Diffuse breast adenosis.**
Left breast **(BIRADS 5): Diffuse breast adenosis with LOQ suspicious mass lesion and ipsilateral pathologically enlarged lymph nodes seen at levels 1,2,3 and supra-clavicular; biopsy of the lesion was taken at same setting.**

Recommendations:

For histopathological correlation and CESM.
Best regards,

Dr. Amal Hegazi, M.Sc.

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Radiology Department

وزارة الصحة والسكان
المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Jihan Abd Al Hamid Mohamed Ahmed	Date : 05/11/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 46 Year(s)
Medical ID: 3547/2024	Pat. ID: 90756

DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

Indication:

Left breast pathologically proven malignant mass

Report:

- Heterogeneous dense fibroglandular parenchymal densities (ACR c)

- Left Breast:
  - Obscured high density spiculated mass is seen at the left LOQ near the inframammary fold. Ultrasound shows it as an irregular hypoechoic infiltrative mass. It is located at 5-6 O'clock, 3mm from the skin measuring about 28x14x20mm.
  - Indeterminate enlarged level I axillary LNs largest measures 10mm in TS diameter and 3.4mm cortical thickness.
  - No pathological enlarged infra or supraclavicular lymph nodes.
- Right Breast:
  - No suspicious micro calcifications or architectural distortion detected.
  - Complementary ultrasound no cystic or solid mass lesions.
  - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
  - Nonspecific axillary LN.

Opinion:

- Left breast pathologically proven malignant mass with ipsilateral indeterminate axillary lymphadenopathy (BIRADS 6)
- Normal sonomammography of right breast (BIRADS 1)

Best regards,
Dr. Amira H Radwan MD,
Breast Imaging consultant,
NCI, Cairo University
05/11/2024 4:04 PM

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Palient Name: Jihan Abd Al Hamid Mohamed Ahmed	Date : 12/05/2025
Exam : MAMMOGRAPHY BOTH	Age : 47 Year(s)
Medical ID: 3547/2024	Pat. ID: 90756

Combined bilateral breast mammography and complementary ultrasound

Indication:

left breast pathgologically proven malignant mass on NACT for assessment.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Dense fibro glandular density (ACR D)
- Left breast shows two outer inner inserted clip with no sizable residual mass lesion BIRADS 6 compared to previous study , current study show good radiological response
- No right breast solid or cystic masses
- Normal skin thickness, and nipple-areola complex of the right breast.
- Nonspecific axillary LN, RIGHT
- Enlarged left axillary LN with preserved fatty hilum
- Free left infra and supra-clavicular regions.

Option:

- Left breast shows two outer inner inserted clip with no sizable residual mass lesion BIRADS 6 compared to previous study , current study show good radiological response.
- Benign findings right breast (BIRADS 2)

Yours truly

Signature Dr . Rehab El Badry

12/05/2025 2:53 PM

Mammogram
[[DOCTYPE:MAMMOGRAM]]